NCCP 00228 · clinician draft

Eribulin · Days 1 and 8 of 21

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Eribulin · Days 1 and 8 of 21 · reviewed supportive care

Active medicines: Eribulin

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Each treatment day only Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- No routine hypersensitivity premedication is specified. Low-risk antiemetic support is reviewed separately, including QT interactions.
- EU dose labels use eribulin base. 1.23 mg/m² base corresponds to 1.4 mg/m² mesylate in some trial reports; these are not two alternative doses.
- Review interactions, QT risk and electrolytes before selecting an antiemetic; individualise rather than automatically issuing this draft.
- Low emetic risk; choose preventive/rescue medicines after considering QT prolongation and interactions. No routine hypersensitivity premedication is usually required.
- Review potassium/magnesium and correct abnormalities; monitor ECG when cardiac/QT risks or symptoms warrant it.
- Severe neutropenia may be managed with G-CSF under the treating team. Avoid driving or machinery when tired or dizzy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sources checked 2026-10-25 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required